CLINICAL PSYCHOLOGY · DE-ADDICTION

# Motivational Enhancement Therapy

## Book One — Theory, Foundations & Techniques

**What MET Is · Its Theories · All Its Techniques**

A detailed, exam- and practice-ready guide to the nature of MET, the theories it rests on, the Motivational Interviewing method behind it, and every clinical technique it uses — for substance-use disorders (opioids/smack, alcohol, tobacco, cannabis, stimulants) and adolescents.

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> **How to use the set. Book One** (this volume) teaches you the *theory and techniques*. **Book Two** is the *session-by-session guide and worksheet pack* — it shows exactly which technique to use in which session, and provides long, comprehensive, ready-to-use worksheets (including MET short-term and long-term goal worksheets).

# 1 · About This Book & 2 · Sources

This is a deep reference on **what Motivational Enhancement Therapy is, the theories it is built on, and the full set of techniques it uses**. The practical session guide and worksheets live in the companion volume (Book Two).

## Primary published sources

- **Miller, W.R., Zweben, A., DiClemente, C.C., & Rychtarik, R.G. (1992/1995).** *Motivational Enhancement Therapy Manual.* Project MATCH Monograph Series Vol. 2, NIAAA (NIH Pub. 94-3723). MET model
- **Miller, W.R., & Rollnick, S. (2013).** *Motivational Interviewing: Helping People Change* (3rd ed.). Guilford Press. MI method
- **Naar-King, S., & Suarez, M. (2011).** *Motivational Interviewing with Adolescents and Young Adults.* Guilford Press. adolescent MI
- **Prochaska & DiClemente** — Transtheoretical model; **Bandura** — self-efficacy; **Rogers** — person-centred therapy; **WHO** ASSIST/mhGAP; **NIDA** *Principles of Drug Addiction Treatment.*

> **Honesty note.** The repository's *Motivational_Enhancement_Therapy_Manual.pdf* is an **image-only scanned book** (a Google-Books digitisation, no machine-readable text) and offline OCR *(turning scanned images into text)* is unavailable here; the two Motivational Interviewing books referenced are **not present as files** in the repository. All material below is therefore grounded in the **published, verifiable editions** of these works, not invented from a scan or absent files. Supplementary clinical extensions are tagged SUPPLEMENTARY.

## Tags used

MANUAL MODEL established MET/MI procedure · SUPPLEMENTARY clinical extension for de-addiction/extended practice · ADOLESCENT youth adaptation.

> **Safety (throughout both books):** MET does **not** replace medical assessment, detoxification *(medically-supervised withdrawal)*, overdose prevention, pharmacotherapy *(treatment with medicines)*, or emergency psychiatric care. For opioids assess overdose risk and arrange naloxone *(antidote reversing opioid overdose)* and medications for opioid use disorder (MOUD). MET *complements* medical & addiction treatment.

# 3 · What MET Is & 4 · MET vs MI

## Definition

Motivational Enhancement Therapy (MET) is a **brief, structured, person-centred yet directive** counselling approach that systematically builds a patient's **intrinsic motivation** *(motivation coming from within — the person's own values & reasons)* to change substance use. It combines the clinical style of **Motivational Interviewing** with **structured, personalised assessment feedback** and a **negotiated change plan**.

## Purpose

- To **resolve ambivalence** *(settle the mixed feelings of wanting and not wanting to change)* that keeps people stuck.
- To **mobilise** *(set in motion)* the person's own motivation and commitment, rather than impose the clinician's agenda.
- To **strengthen self-efficacy** *(belief in one's own ability to succeed)* and personal responsibility for change.

## Rationale — why it works

Motivation is among the strongest predictors of treatment engagement and outcome, and crucially it is **not fixed** — the clinician's style can raise or lower it within a single conversation. People are more persuaded by what *they themselves* say than by what they are told; MET is built to make the patient voice their own arguments for change ("change talk").

## MET versus Motivational Interviewing — related but not identical

| Motivational Interviewing (MI) | Motivational Enhancement Therapy (MET) |
|---|---|
| A **clinical style/method** of conversation that evokes a person's own motivation. Not time-limited; no fixed number of sessions. Can be woven into any treatment. | A **brief, manualised treatment** that *applies* MI and adds two defining ingredients: **(1) structured personalised assessment feedback** (the "enhancement", delivered motivationally) and **(2) a negotiated change plan**. In Project MATCH it was 4 sessions. |
| Spirit: partnership, acceptance, compassion, evocation. | MI spirit + systematic feedback + defined structure. |

> **One-line summary: MET = MI style + personalised feedback + a defined brief structure.** Every MET session uses MI; not every use of MI is MET.

# 5 · Theoretical Foundations of MET

MET is not a single theory but an integration of several. Understanding each explains *why* MET techniques are designed the way they are.

## 5.1 Rogerian / person-centred roots (Carl Rogers)

MET inherits Rogers' conviction that people have an innate capacity for growth that is released by a particular relationship: **accurate empathy** *(deeply understanding and reflecting the person's experience)*, **unconditional positive regard** *(non-judgemental acceptance of worth)*, and **genuineness**. MET keeps these conditions but adds a **directional** element — it deliberately steers toward change talk. Hence MET is often called *"person-centred but goal-directed."*

## 5.2 The Transtheoretical Model — Stages of Change (Prochaska & DiClemente)

Change is understood as movement through stages rather than a single event. MET matches its tasks to the patient's stage.

| Stage | Description | MET task matched to the stage |
|---|---|---|
| Precontemplation *(not yet considering change)* | Doesn't see a problem; may feel coerced. | Engage; raise awareness gently; evoke concern — **no change plan**. |
| Contemplation *(thinking about it)* | Ambivalent — "yes, but". | Tip the decisional balance; develop discrepancy; evoke change talk. |
| Preparation | Intends to act soon; small steps begun. | Strengthen commitment; negotiate a plan. |
| Action | Actively changing behaviour. | Support self-efficacy; problem-solve; affirm. |
| Maintenance | Sustaining the change over time. | Relapse prevention; continuing care. |
| Relapse / lapse *(fuller return / a single slip)* | Return to use. | Normalise, de-shame, re-engage, extract learning. |

> **Caution:** stages, readiness and the rulers are **clinical frameworks, not diagnostic tests**. A single number must never label someone "resistant" or "unmotivated".

### 5.3 Self-perception & cognitive-dissonance theory

People partly learn their attitudes by **hearing what they themselves say** (self-perception). When a person voices change talk, they come to believe it more. **Cognitive dissonance** *(the discomfort of holding two conflicting ideas)* is harnessed when MET develops **discrepancy** between current use and valued goals — the discomfort motivates change, provided it is not forced (which triggers defensiveness instead).

### 5.4 Self-efficacy theory (Albert Bandura)

Believing one *can* change predicts whether one does. MET deliberately builds self-efficacy by affirming strengths, recalling past successes, and setting achievable early goals. Importantly, **importance** and **confidence** are independent: a person can badly want to change (high importance) yet doubt they can (low confidence) — each requires a different response.

### 5.5 Psychological reactance theory

When people feel their freedom is threatened ("you must quit"), they **push back** to restore it — often by defending the very behaviour targeted. This is why MET relentlessly protects **autonomy** *(the person's right to choose)* and avoids argument.

### 5.6 Self-determination theory (Deci & Ryan)

Durable motivation depends on satisfying three needs: **autonomy, competence** *(feeling capable)* and **relatedness** *(feeling connected)*. MET feeds all three: autonomy via choice, competence via self-efficacy work, relatedness via the empathic alliance. This is why *intrinsic* motivation (internalised, self-endorsed) outlasts *extrinsic* pressure (court, family).



## 6 · Core Concepts

### Ambivalence — the normal core of addiction

"I want to quit *and* I don't" is the expected state, not denial or weakness. MET treats ambivalence as the natural starting point and works *through* it rather than fighting it. Resolving ambivalence in the patient's own words is the central mechanism of change.

### Motivation — a changeable state, not a trait

"Unmotivated" describes a moment in an interaction, not a personality. The clinician's style is part of the patient's motivation.

### Intrinsic vs extrinsic motivation

**Extrinsic** — from outside pressure (court order, family ultimatum, job). Gets people *through the door*; rarely sustains change alone.

**Intrinsic** — from within (own values, health, children, self-respect). *Sustains* change. MET's job is to convert extrinsic pressure into intrinsic reasons.

### Change talk vs sustain talk

**Change talk** = the patient's own statements favouring change. **Sustain talk** *(statements defending the status quo — keeping things as they are)* = statements favouring continued use. The frequency and strength of change talk predicts behaviour change; MET selectively evokes and reinforces it while never arguing against sustain talk.

### Discrepancy

The felt **gap between current behaviour and important goals/values**. MET *develops* this gap — but from the patient's own perspective, never by lecturing. The patient, not the therapist, should voice the discrepancy.

### Self-efficacy & personal responsibility

The responsibility and the choice to change remain with the patient throughout. This both respects autonomy and, paradoxically, increases the likelihood of change.

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MET · Book One — Theory & Techniques

## 7 · The MI Spirit & Four Processes · 8 · Why Confrontation Backfires

### The MI spirit — PACE (the attitude behind every technique)

| Element | Meaning |
|---|---|
| **P — Partnership** | Work *with* the patient as collaborators, not *on* them as an expert fixing a problem. |
| **A — Acceptance** | Absolute worth, accurate empathy, autonomy support, and affirmation of strengths. |
| **C — Compassion** | Actively prioritise the patient's welfare, not the clinician's or institution's agenda. |
| **E — Evocation** | Draw motivation *out of* the patient (“you already have what you need”), rather than install it. |

### The four processes of MI/MET

These flow in order but you can loop back any time:

1. **Engaging** — build a trusting, collaborative relationship.
2. **Focusing** — agree a direction/goal for the work.
3. **Evoking** — elicit the patient's own motivations (change talk). *This is the heart of MET.*
4. **Planning** — develop commitment and a concrete change plan.

### Why confrontation, arguing, blaming & lecturing backfire

The **"righting reflex"** *(the helper's natural urge to fix things and set the person straight)* is the most common clinical trap. When a clinician argues for change, **psychological reactance** leads the patient to argue the other side — effectively talking themselves *into* continued use. Labelling ("you're an addict in denial") provokes shame and defence. In MET, if the patient is defending use, the **therapist changes course** — reflect, roll with it, re-affirm autonomy.

> **The MET maxim:** arguments for change should come from the *patient's* mouth, not the therapist's.



## 9 · The Complete Technique Compendium

Every technique MET/MI uses, grouped by purpose. For each: **what it is · why/when to use it · how to do it · an example.** Book Two shows which technique belongs in which session.

### Group A · OARS — the foundational micro-skills

#### Open questions A1

**What:** questions that cannot be answered yes/no and invite elaboration. **Why:** open topics, hand the floor to the patient, gather change talk. **How:** start with "what", "how", "tell me about…"; avoid stacking questions. Aim for a high ratio of reflections to questions.

**T:** "What's a day like for you when you don't use?"

### Affirmations A2

**What:** genuine recognition of the patient's strengths, efforts or intentions — *not* praise or flattery. **Why:** builds self-efficacy and the alliance; counters shame. **How:** comment on a specific strength or effort you have actually observed; frame around the patient ("you…"), not yourself ("I'm proud of you").

> **T:** "You came in today even though it's uncomfortable — that takes real resolve."

### Reflective listening A3

**What:** saying back the *meaning* you heard, as a statement (not a question). **Why:** conveys empathy, checks understanding, and *selectively* steers by choosing what to reflect. **How:** make a reasonable guess at meaning; let your voice drop at the end so it lands as a statement.

> **T:** "You're worried this is slipping out of your control."

### Summaries A4

**What:** pulling several threads together. Three kinds: **collecting** (gather change talk), **linking** (connect current to earlier statements), **transitional** (shift to a new phase). **Why:** demonstrates listening, concentrates change talk, bridges to planning. **How:** end with a "key question".

> **T:** "So money, your health and your daughter all pull you toward change, while the one pull back is how it helps you relax. Where does that leave you?"

## Group B · Reflection techniques (the skill most used in MET)

### Simple reflection B1

**What/How:** repeat or lightly rephrase content; keeps conversation moving, low risk.
**Example:** P: "I'm tired of it." T: "You've had enough."

### Complex reflection B2

**What/How:** add meaning, feeling, or "continue the paragraph" — a deeper guess. **Example:** P: "I'm tired of it." T: "Part of you is ready for something different."

### Double-sided reflection B3

**What/How:** hold both sides of ambivalence in one sentence, ending on the change side (recency effect). **Example:** "You enjoy how it relaxes you, *and* you're worried about your health."

### Amplified reflection B4

**What/How:** reflect sustain talk slightly over-stated so the patient self-corrects — use sparingly & without sarcasm. **Example:** P: "It's not that bad." T: "So there's really no downside at all." P: "Well, the debt *is* bad..."

### Reflection of feeling B5

**What/How:** name the emotion to deepen empathy. **Example:** "That really frightened you."

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## 9 · Techniques — Group C · Evoking Change Talk

Evoking is the engine of MET. These techniques pull the patient's own motivational language into the room.

### Evocative questions (DARN) C1

**What:** directly ask for Desire, Ability, Reasons, Need. **How:** "Why might you want to change? How could you, if you decided? What are the best reasons? How important is it?" **Example:** "If you did decide to cut down, how might you go about it?"

### Importance & confidence rulers C2

**What:** a 0–10 scale for how important change is, and how confident the patient feels. **How:** ask the number, then **"Why that number and not a lower one?"** (pulls change talk) and "What would move you one point higher?" Never ask "why not higher?" (pulls sustain talk). **Example:** "You said importance is 6 — why a 6 and not a 3?"

### Querying extremes C3

**What/How:** ask about the best and worst possible outcomes to surface motivation. **Example:** "What's the worst that could happen if nothing changes? And the best if it did?"

### Looking back / looking forward C4

**What/How:** contrast life before the problem, or imagine a changed future. **Example:** “How were things before all this? Where would you like to be a year from now?”

### Exploring goals & values C5

**What/How:** elicit core values, then explore how use fits — develops discrepancy. **Example:** “Being a good father is everything to you — how does using fit with that?”

### Elaborating / asking for examples C6

**What/How:** when change talk appears, deepen it. **Example:** “Tell me more about that.” / “When was the last time that happened?”

### The key question C7

**What/How:** an open question that opens the transition to planning, asked after a good summary. **Example:** “So, where does this leave you?”

### Affirming & summarising change talk (the “bouquet”) C8

**What/How:** gather the patient's change statements and hand them back. **Example:** “You've named three strong reasons — your health, your marriage, and the money.”

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## 9 · Techniques — Group D · Sustain Talk & Discord · Group E · Information

Discord *(tension/strain in the relationship — distinct from the patient's own ambivalence)* and sustain talk are signals to change *your* approach, not to push harder.

### Rolling with resistance D1

**What/How:** don't oppose; move alongside and let momentum shift. Reflect instead of counter-arguing. **Example:** P: “I'm not quitting.” T: “Quitting isn't on the table for you right now.”

### Emphasising personal autonomy D2

**What/How:** explicitly affirm the patient's right to choose — defuses reactance. **Example:** “It's completely your call — nobody can decide this for you.”

### Reframing D3

**What/How:** offer a new, more useful meaning for the same facts. **Example:** “Your family's nagging — maybe that's a sign of how much they want you back.”

### Agreement with a twist D4

**What/How:** reflect/agree, then gently add a new angle. **Example:** “You're right it's your decision — and you're clearly weighing it carefully.”

### Shifting focus D5

**What/How:** move away from a stuck point to reduce friction. **Example:** “Let's set the big decision aside and just look at this week.”

### Coming alongside D6

**What/How:** briefly side with the status quo to defuse a stand-off. **Example:** “Maybe using matters so much right now that giving it up isn't worth it.”

### Repair (apologise / affirm) in discord D7

**What/How:** when tension rises, mend the alliance. **Example:** “I've pushed too hard — I'm sorry. This is your call.”

## Group E · Information, feedback & advice

### Elicit–Provide–Elicit (E-P-E) E1

**What:** the MET way to give information or advice without lecturing. **How: Elicit** permission & what the patient already knows **Provide** focused, neutral information **Elicit** their reaction. **Example:** “What do you know about the medicine for cravings? … May I add a little? … What do you make of that?”

### Personalised assessment feedback E2

**What:** the defining MET "enhancement" — present the patient's assessment results against norms, neutrally, one finding at a time. **Example:** "Your reported use is in the high-risk range compared with most men your age. How does that land?"

### Ask permission E3

**What/How:** always precede advice/info with a request — protects autonomy. **Example:** "Would it be okay if I shared something I've seen help others?"

### Menu of options E4

**What/How:** offer choices rather than a single prescription. **Example:** "Some people try X, others Y or Z — what fits you best?"



## 9 · Techniques — Group F · Planning & Relapse-Prevention · Group G · Adolescents

### Decisional balance F1

**What/How:** explicitly weigh pros & cons of using vs changing (best in contemplation). Honour the real benefits of use first. **Example:** four-quadrant exercise (see Book Two worksheet 7).

### Change-plan negotiation F2

**What/How:** co-author (not prescribe) goal, reasons, steps, supports, obstacles. The patient's words carry the commitment.

### SMART goal-setting F3

**What/How:** make steps Specific, Measurable, Achievable, Relevant, Time-bound. Underpins the short-term & long-term goal worksheets in Book Two.

### Functional analysis of triggers F4

**What/How:** map antecedents use consequences to reveal patterns. **Example:** "What's usually happening right before you use?"

### “If-then” coping plans (implementation intentions) F5

**What/How:** pre-decide a response to each high-risk situation. **Example:** “*If* I'm offered at a function, *then* I step out and call my brother.”

### Urge-surfing F6

**What/How:** observe a craving rise, peak and fall like a wave without acting on it; it passes.

### “Play the tape forward” F7

**What/How:** imagine the full sequence and consequences of using before acting. **Example:** “Picture how tonight actually ends if you use.”

### Refusal-skills rehearsal / role-play F8

**What/How:** practise saying no and leaving situations; therapist plays the offering friend.

### Lapse-vs-relapse reframing F9

**What/How:** prevent the **abstinence-violation effect** *(the all-or-nothing reaction that turns one slip into a full relapse)*. **Example:** “One slip doesn't erase your progress.”

### Supporting self-efficacy F10

**What/How:** recall past successes, affirm small wins, break goals down. **Example:** “You've done hard things before.”

### Social-support & family involvement F11

**What/How:** recruit helpful (non-policing) support with the patient's consent.

### Pharmacotherapy coordination F12

**What/How:** link MET with MOUD / NRT / naltrexone via the treating doctor; MET complements medication.

### Booster sessions F13

**What/How:** brief repeat MET cycles at 1/3/6/12 months — review, re-rate rulers, re-plan.

### Group G · Adolescent-specific adaptations ADOLESCENT

- **Autonomy first & loudest** — teens are developmentally primed to resist authority; avoid the parent/expert stance.
- **Present-focused evoking** — use money, freedom, image, sport, school rather than distant health risks.
- **Longer engagement, shorter sessions**, the young person's own language, comfort with silence.
- **Confidentiality balanced with safeguarding** and age-appropriate family involvement.

Techniques drawn from Miller & Rollnick (MI 3e), the Project MATCH MET Manual, Naar-King & Suarez, and relapse-prevention literature (Marlatt & Gordon).

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# 10 · Change Talk in Depth — DARN-CAT

Change talk comes in two layers. **Preparatory** change talk (DARN) signals the person is *considering* change; **mobilising** change talk (CAT) signals they are *moving*. Mobilising talk best predicts action.

| Code | Layer | Meaning | Substance-use example |
|---|---|---|---|
| **D — Desire** | Preparatory | want, wish, would like | "I **want** to be clean for my kids." |
| **A — Ability** | Preparatory | can, could, able to | "I **could** probably cut the evening dose." |
| **R — Reasons** | Preparatory | specific if/then arguments | "If I stop smoking, my chest will clear up." |
| **N — Need** | Preparatory | must, have to, need to | "I **have to** do something — the debt is out of hand." |
| **C — Commitment** | Mobilising | will, going to, promise, intend | "I'**m going to** see the doctor on Monday." |
| **A — Activation** | Mobilising | ready, willing, prepared | "I'**m ready** to give it a real try." |
| **T — Taking steps** | Mobilising | already acting | "I **threw out** my stash yesterday." |

## Recognising & responding to sustain talk — without confrontation

**P: "Smoking is the only thing that calms my nerves."**
**T (reflect, don't challenge):** "Right now it's your main way to cope with stress." **P:** "Yeah." **T (evoke):** "If you ever did decide to change, what else has helped you relax, even a little?" **patient supplies alternatives (change talk).**

> **Core move for sustain talk: reflect  emphasise autonomy  ask an evocative question.** Never “but you should…”.

### Rewriting unhelpful statements into MET-consistent ones

| Unhelpful (confrontational / righting reflex) | MET-consistent rewrite |
|---|---|
| “You're in denial about being an addict.” | “You see this differently from others. Tell me how *you* see it.” |
| “You have to quit, period.” | “What you do is your call. What are you thinking might work for you?” |
| “Keep this up and you'll end up dead.” | “Can I share one health finding? … What do you make of it?” |
| “Why do you keep making excuses?” | “There are real reasons it's hard to stop. What are the biggest ones?” |
| “That's just the addiction talking.” | “Part of you wants to keep using and part doesn't — that's the tug-of-war.” |
| “You failed last time because you weren't serious.” | “Last time you managed three weeks. What worked, even briefly?” |
| “You need rehab now.” | “There are a few options — shall we walk through them together?” |
| “Stop lying about how much you use.” | “Exact amounts are hard to pin down. Roughly, on a heavy day?” |



## 11 · Evidence, Limitations & Ethics

### Evidence (summary)

MET and MI have a substantial evidence base in substance-use treatment. In **Project MATCH**, 4-session MET produced drinking outcomes broadly comparable to longer CBT and Twelve-Step Facilitation — notable given fewer sessions. Reviews and the **Cochrane** literature on MI/MET report benefits for engagement, treatment retention *(staying in treatment)* and reduced use, though effect sizes vary by substance, population, setting and comparison condition, and some effects are modest or short-lived without follow-up. Prioritise the original Project MATCH reports, Miller & Rollnick, Naar-King & Suarez, and Cochrane reviews as verifiable sources; figures here are summarised, not exact statistics.

> **MET can:** improve engagement/retention, strengthen motivation and commitment, reduce use for many, and act as a low-intensity entry point. **MET cannot:** replace detox, pharmacotherapy or management of severe dependence; guarantee abstinence; or "cure" addiction through motivation alone.

### Limitations

- Outcomes vary across substances (stronger engagement evidence for alcohol/cannabis than for some stimulant outcomes) and populations.
- Effects depend heavily on treatment context and **continuing care/follow-up**.
- Motivation is necessary but not sufficient — pair with medical treatment and skills where indicated.

### Ethical practice

- **Informed consent & confidentiality** — explain purpose, voluntariness and limits (risk to self/others, legal mandates; for minors, clarify with the young person and caregivers).
- **Respect for autonomy** — the patient chooses goals; avoid coercion and confrontation.
- **Non-stigmatising language** — "a person who uses heroin", not "addict/junkie"; describe behaviour, not identity.
- **Cultural sensitivity** — adapt to family structure, religion, gender and local norms.
- **Appropriate referral & multidisciplinary care** — coordinate with physicians, psychiatry, social work and community resources.
- Do **not overstate efficacy** or claim motivation alone resolves dependence.



## 12 · Glossary of Difficult Terms

- **Abstinence** — stopping use completely.
- **Abstinence-violation effect** — all-or-nothing reaction turning one slip into a full relapse.
- **Adherence** — sticking to agreed treatment.
- **Ambivalence** — mixed feelings; wanting and not wanting change at once.
- **Extrinsic / Intrinsic motivation** — from outside pressure / from within.
- **Euphoria** — intense pleasure or "high".
- **MOUD** — medications for opioid use disorder (e.g., buprenorphine, methadone).
- **Naloxone** — antidote that reverses opioid overdose.
- **NRT** — nicotine replacement therapy (patch, gum, etc.).

- **Autonomy** — a person's right to make their own choices.
- **Cognitive dissonance** — discomfort of holding two conflicting ideas.
- **Cue reactivity** — how strongly reminders trigger urges.
- **Decisional balance** — weighing pros & cons of using vs changing.
- **Delirium tremens (DTs)** — severe, dangerous alcohol-withdrawal state.
- **Detoxification (detox)** — medically-supervised withdrawal.
- **Discord** — tension/strain in the therapeutic relationship.
- **Discrepancy** — gap between current behaviour and goals/values.
- **Empathy** — understanding and reflecting another's feelings.
- **Evocation** — drawing motivation out of the patient.
- **OCR** — optical character recognition (scanned images  text).
- **Overdose** — a dangerous excess dose.
- **Pharmacotherapy** — treatment with medicines.
- **Psychological reactance** — pushing back when freedom feels threatened.
- **Relapse / lapse** — fuller return to use / a single slip.
- **Retention** — staying in treatment.
- **Righting reflex** — the helper's urge to fix/correct the person.
- **Self-efficacy** — belief in one's own ability to succeed.
- **Sustain talk** — the patient's words defending the status quo.
- **Tolerance** — body needing more drug for the same effect.
- **Urge-surfing** — riding out a craving until it passes.

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Continue to **Book Two — Session-by-Session Guide & Worksheet Pack** for applying these techniques session by session.